

Hospital Readmission Risk: Business Impact & Strategic Recommendations

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Hospitals Over Benchmark 48.15% National Medicare threshold	Highest Ratio State MA Ratio: 1.034	Lowest Ratio State ID Ratio: 0.943	Top Readmission Cause Pneumonia 23% of cases
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BUSINESS IMPACT

Medicare Penalty Risk

Nearly half of hospitals (48.15%) have exceeded the national benchmark, placing them at significant risk of **Medicare financial penalties** under the Hospital Readmissions Reduction Program (HRRP). Left unaddressed, this translates directly to reduced reimbursement and reputational risk.

Regional Performance Gap

A wide **regional performance gap** exists — Massachusetts (1.034) versus Idaho (0.943) — indicating that geography, care coordination infrastructure, and post-discharge support vary significantly across markets.

Ownership Model Impact

Government-Federal and Proprietary hospitals carry the highest readmission risk, while Physician-Owned institutions demonstrate best-in-class outcomes — suggesting ownership structure and operational model are key performance drivers.

Pneumonia as Primary Driver

Pneumonia accounts for 23% of readmissions, making it the single largest clinical driver and the highest-leverage condition for targeted quality improvement investment.

STRATEGIC RECOMMENDATIONS

1

Deploy Predictive Readmission Models

Integrate AI-driven risk stratification tools into discharge workflows to proactively flag high-risk patients before they leave the facility.

2

Strengthen Discharge & Post-Acute Follow-Up

Establish structured post-discharge call programs and accelerate outpatient appointment scheduling within 7 days of release to close the care gap.

3

Focus Resources on Highest-Risk Markets

Massachusetts, New Jersey, Florida, and Illinois require immediate quality improvement initiatives and may benefit from additional care management staffing and patient education programs.

4

Adopt Best Practices from Top-Performing States

Idaho's low readmission ratio (0.943) reflects strong care coordination models worth formalizing into a national benchmarking framework for system-wide adoption.

5

Target Care Management in Higher-Risk Ownership Groups

Government-Federal and Proprietary hospitals should prioritize investment in discharge support programs, transitional care teams, and case management infrastructure.